

Kinora Head & Face

Clinical AI Orientation (Part 15)

Facial Muscles, Cranial Nerves, Headache, Concussion & Vestibular Basics

Version 1.0 — Kinora Admin Conocimientos Upload

Disclaimer

Educational only. Sudden severe headache, focal neurologic deficit, suspected stroke/CAD, or deteriorating concussion — emergency care.

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1. Facial Muscles (Overview)

RECORD: Facial Expression Muscles

ID: HEAD-MUS-01

Innervation: CN VII facial nerve

Clinical: Bell's palsy vs central facial palsy sparing forehead

References: Standring S. Gray's Anatomy. Moore KL et al. Clinically Oriented Anatomy. Netter FH. Neumann DA. Kinesiology of the Musculoskeletal System. Magee DJ. Orthopedic Physical Assessment. Brukner & Khan Clinical Sports Medicine.

RECORD: Occipitofrontalis / Occipitalis

ID: HEAD-MUS-02

Function: Scalp/forehead; tension-type headache contributor

References: Standring S. Gray's Anatomy. Moore KL et al. Clinically Oriented Anatomy. Netter FH. Neumann DA. Kinesiology of the Musculoskeletal System. Magee DJ. Orthopedic Physical Assessment. Brukner & Khan Clinical Sports Medicine.

2. Cranial Nerves — MSK Relevance

RECORD: CN Summary for Physio

ID: HEAD-CN-01

CN_II_III_IV_VI: Vision/eye movements — dizziness/diplopia red flags

CN_V: Facial sensation; muscles of mastication (V3); neuralgia

CN_VII: Facial expression; corneal reflex efferent

CN_VIII: Hearing/vestibular — BPPV/vestibular neuritis differentials

CN_XI: SCM/trapezius

Screen: Any cranial nerve deficit after trauma/neck pain — urgent workup (CAD/central)

References: Standring S. Gray's Anatomy. Moore KL et al. Clinically Oriented Anatomy. Netter FH. Neumann DA. Kinesiology of the Musculoskeletal System. Magee DJ. Orthopedic Physical Assessment. Brukner & Khan Clinical Sports Medicine.

3. Headache Classifications (Clinical Framing)

RECORD: Primary vs Secondary

ID: HEAD-HA-01

Primary: Migraine, tension-type, cluster (IHS ICHD frameworks)

Secondary_MS: Cervicogenic; TMD-related

Red_Flags_SNNOOP10: Systemic, Neurologic, Onset sudden, Older, Pattern change, Positional, Precipitated by Valsalva, Papilledema, Progressive, Pregnancy — refer

References: ICHD headache classification themes; Standring S. Gray's Anatomy. Moore KL et al. Clinically Oriented Anatomy. Netter FH. Neumann DA. Kinesiology of the Musculoskeletal System. Magee DJ. Orthopedic Physical Assessment. Brukner & Khan Clinical Sports Medicine.

4. Concussion Screening Basics

RECORD: Concussion Orientation

ID: HEAD-CONC-01

Definition: Traumatic brain injury induced by biomechanical forces (Berlin/Amsterdam consensus themes)

Screen: SCAT-type symptom/cognitive/balance tools in trained settings; remove from play if suspected

Red_Flags: Declining consciousness, seizures, focal neuro — emergency

Return_to_Sport: Graduated RTS protocols after rest/symptom-limited activity per current consensus

References: Concussion in Sport consensus statements; Standring S. Gray's Anatomy. Moore KL et al. Clinically Oriented Anatomy. Netter FH. Neumann DA. Kinesiology of the Musculoskeletal System. Magee DJ. Orthopedic Physical Assessment. Brukner & Khan Clinical Sports Medicine.

5. Vestibular Basics

RECORD: Vestibular Orientation for MSK Clinicians

ID: HEAD-VEST-01

Peripheral: BPPV (canalithiasis) — Dix-Hallpike/CRM; vestibular neuritis

Central_Red_Flags: HINTS battery by trained clinicians; vertical skew, dangerous nystagmus, acute hearing loss patterns — urgent

Cervicogenic_Dizziness: Diagnosis of exclusion after vascular/vestibular/central screen

References: Standring S. Gray's Anatomy. Moore KL et al. Clinically Oriented Anatomy. Netter FH. Neumann DA. Kinesiology of the Musculoskeletal System. Magee DJ. Orthopedic Physical Assessment. Brukner & Khan Clinical Sports Medicine.